Ethics Relating to the Retention and Display of Human Remains in Medical Museums: Case Studies at the Surgeons' Hall Museums, Edinburgh

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'They tell us it was necessary for science. Science? Why, who is science for? Not for poor people. Then if it is necessary for science, let them have the bodies of the rich, for whose benefit science is cultivated.'[1]

[1] William Cobbett, 1830, 13, commenting upon the proposed Anatomy Act.

Introduction

Current literature on ethics of retaining and displaying human remains (HR) within museums largely centres around repatriation and colonial violence.[2] I accept when one community with power takes HR from another community (either dispossessed or in another geographical location), it is ethically wrong, and repatriation entails appropriate ethical action. What changes when the harmful power relationships are contained within the same community? I explore this using the Surgeons' Hall Museums (tSHM) in Edinburgh, given Edinburgh's rich medical history and as tSHM contain relatively high numbers of medicalised HR from its relatively small locality of Edinburgh. HR within medical museums, and the results of dissection within historical Edinburgh, have been largely underdiscussed.[3] The Department of Culture, Media and Sport in the United Kingdom (DCMS) Guidance on Human Remains suggests all HR should be treated with 'respect and dignity' - highly charged terms often not further explicated.[4] Given much of the relevant literature does not address general ethical frameworks, this essay will be an explorative and perhaps 'informal' account into ethics of HR within medical museums that emphasises the role of history.[5] For clarity, I refer to persons whose HR are under discussion as 'the individual', and persons responsible for HR (e.g. museum staff – this is the extent I discuss responsibility) as 'the agent'. After preliminary analysis of relevant ethical principles, I illustrate the significance of history for making ethical judgements around HR, given the importance of belief, ties with living communities, and historical power relations. I then provide relevant historical details, which I use in case studies highlighting impacts of Edinburgh's criminal justice system, class, and gender. Ultimately, I conclude museum practitioners at tSHM should revise language used to describe HR and provide better historical contextualisation. This allows tSHM to fulfil roles as medical, historical, and moral educators, which best justifies retaining and displaying the HR in their collections.

[2] Alberti, 2011, 11. Clegg, 2020, 136. This is colonial violence broadly construed. Various contexts have sparked further literature, such as HR from Holocaust victims or the Sami in Sweden (e.g. Aly et. al., 2017. Mulk, 2009. Svestad, 2019).

[3] Alberti, 2011, 11, 69-70.

[4] Barilan, 2006, 234-246. Clegg, 2020, 44. It is beyond this essay to explicitly develop an account of these concepts.

[5] Clegg, 2020, 3-4. By 'informal', I utilise Stroll's definition whereupon I emphasise common sense, appeal to ordinary discourse, and highly contextualised lines of enquiry (Stroll, 2009).

Ethics Regarding HR

It is a widely discussed ethical principle that using a person as a means to an end is morally wrong.[6] Given this, is it morally wrong to use a dead person as a means to an end – say, for education and research which could save lives - particularly when in contradiction to the individual's wishes while alive, or without consent? If the dead do not have features presupposed for personhood, this principle may not apply.[7] One could argue the dead cannot be harmed or immorally used as means-to-ends as they cannot know what happens to them, and cannot have moral rights or interests as they cannot gain or lose the 'goods of life'.[8]

Within medical ethics, rights to jurisdiction over one's own body can provide important foundations for answering questions about consent and medical research.[9] Barilan has argued the concept of informed consent (which I do not attempt to define further) rests on presuppositions of moral duties to the dead (MDD).[10] Based on his account, I accept we have MDD.[11] I explore MDD either on account of the dead, as an extension of moral treatment of property, or on account of the living. If one accepts these premises, it may be unethical to retain or display HR in museums when in contradiction to the individual's living wishes.

Do we own our bodies? Rights of an individual to assert what happens to their property after death, and some degree of agents' duties to carry out their wishes, are commonly accepted. If we understand HR as the individual's property, this extends to HR.[12] As with personal property, HR could thus either be willed to museums or transition to cultural property, particularly if time has passed.[13] Unlike cases of colonialism, HR kept by descendants of cultures they lived within (as at tSHM) would thus not invoke ethical ramifications of theft. Museum practitioners

[6] See Kleingeld, 2020 for this as an element of Kantian moral theory. This principle is applied in various realms of ethics; particularly relevant are discussions on harvesting organs from HR of non-consenting individuals (e.g. Altman, 2011. Merle, 2000. Powers, 1999. Van der Graaf and Van Delden, 2012).
[7] See Baker, 2000 and Wiggins, 1980 for discussions of personhood and the body.
[8] Barilan, 2006, 240, 244.
[9] Alberti, 2011, 207. Consent raises further issues with regards to HR in medical museums (e.g. consenting to donate one's body to science after death is not necessarily consent for one's HR to be public displayed).
[10] Barilan, 2006, 233, 241.
[11] See Barilan, 2006 for a more developed account.
[12] Alberti, 2011, 100-101. See Björkman and Hansson, 2006 for an exploration of property rights within bioethics.
[13] Clegg, 2020, 52. Current UK legislation covers HR from the last 100 years. This may be a plausible timeframe (if somewhat arbitrary) within which personal property becomes cultural property, perhaps akin to intellectual property.

sometimes discuss HR as property. For example, the 2009 Anthology on tSHM refers to HR as 'gifts', implying ownership transferred from the individual to the museum.[14] Perhaps this reflects their legal ownership and duties to care for HR.[15] However, treating HR merely as property does not seem to correspond to full, intuitive and emotive feelings of having MDD.[16]

Even if one rejected the earlier claim of certain MDD as the dead have no moral rights or interests, one might still argue for MDD in the moral interest of the living. Death is highly meaningful to human beings, and it is common to desire control over one's body after death.[17] When an agent respects an individual's wishes, perhaps they act in the moral interests of other living persons by enacting a reassuring social contract. Treatment of HR also concerns living persons with personal, familial or cultural ties to the dead. Repatriation cases are the most obvious examples, where respecting and understanding beliefs of Indigenous communities is significant in deciding when to repatriate HR.[18] This suggests HR should be treated differently given the beliefs of the relevant living communities.

Moral emphasis on living communities is reflected in temporal components of current UK legislation and guidance on HR in museums. DCMS guidelines, which are generally well-received amongst museum practitioners, identify four temporal categories: the newly dead ('deaths among relatives of living people'), the recently dead ('those whose identities we may know and of whom we may have intergenerational knowledge'), the long dead ('deaths from a time when we are less likely to have any knowledge of the individual's personal identity'), and the ancient dead.[19] The first two categories have the most legal protection. The older the remains, the weaker the personal, familial and cultural ties, making this a pragmatic, general approach for addressing the number of living persons who might be harmed by HR in museums.[20] It is perhaps a somewhat strange consequence that harm caused by using HR for research and display could change over time, but not entirely implausible.

[14] Kemp, 2009, 7.
[15] I return to object-based language at SHM later in the essay.
[16] For example, associated concepts of rights to sell our bodies or organs seem problematic.
[17] Barilan, 2006, 238.
[18] Ibid., 235. Clegg, 2013, 51. Clegg, 2020, 86-87, 110.
[19] Clegg, 2020, 46, 55, 60.
[20] Ibid., 65.

From these considerations, I establish a history-based ethical framework through which to think about HR. Firstly, in order to fulfil MDD in following an individual's wishes, their probable wishes must be understood to the best degree possible, based upon society and culture within their period and location. Certain epistemic issues arise when attempting to attribute beliefs to HR, but using Barilan's account I generally assume a plausible degree of certainty is sufficient for action of the agent, which I return to later, and that cultural and historical considerations are reasonably suggestive of an individual's own wishes.[21] Secondly, history provides important information on relations of HR to the living, as harm to living persons depends on cultural, familial and personal ties. Finally, history can tell us about harmful treatment of HR in the past, which I will argue affects how tSHM should treat HR in the present.

Historical Context

I only present a brief history of Edinburgh and tSHM based on what is most relevant to ethical analysis. Edinburgh was a Christian society, and a widespread belief within the 1700s to 1900s was that the body must be buried intact for resurrection at the Rapture.[22] Disposal of the dead was covered by canon law under the Church, requiring burial in consecrated ground other than for deaths via suicide or capital punishment.[23] For centuries, the Church forbade dissection as it precluded resurrection, which led to dissection being used for (seemingly retributionist) punishment of convicted criminals.[24] This can also be understood as a deterrent; while perhaps strange to modern audiences, many criminals were reportedly more afraid of post-mortem dissection than execution.[25]

By the early 1800s, surgeons desired a much greater number of bodies, leading to graverobbing and the infamous case of William Burke and William Hare who in 1828 Edinburgh committed murder for the purpose of selling their victims' bodies to anatomists.[26] Other common practices of non-consensual dissection and tissue collection caused many poor people to fear hospitals.[27]

[21] Barilan, 2006, 238-239.
[22] Alberti, 2011, 70-71. Clegg, 2020, 11. Dacome, 2006, 74, 81.
[23] Clegg, 2020, 46. Daniell, 1997.
[24] Alberti, 2011, 70-71. Clegg, 2020, 11, 19. Dacome, 2006, 103. Dittmar and Mitchell, 2016, 715. Ward, 2015, 63-64. Retributionism in the ethics of punishment has resurged in popularity within penological and philosophical writing in recent decades (Ellis, 1995, 225-226. Ryberg, 2004, 4-5).
[25] Bennett, 2017, 159.
[26] Barilan, 2006, 236. Dittmar and Mitchell, 2016, 715. Mitchell et al., 2011, 91-92. SHM, Edinburgh. Ward, 2015, 63-68, 71.
[27] Clegg, 2020, 19-20. Dittmar and Mitchell, 2016, 714.

The 1832 Anatomy Act, partly arising due to the Burke and Hare murders, ruled 'unclaimed' bodies from public institutions like workhouses, hospitals and prisons should be used instead of executed criminals.[28] The claimant process did not require mandatory notice to be given to the deceased's family, but rather specified any person willing to pay for burial could take a body within a certain timeframe after death. Bodies were thus sometimes sold by impoverished family members or given to medical institutions on the caveat they pay for burial, examples of second party consent given under duress.[29] While intended to prevent graverobbing, the Act mainly (adversely) affected the poor. It specified bodies should be buried after eight weeks 'with the observances peculiar to the religious creed of the deceased'.[30] However, it did not explicitly refer to keeping HR within medical collections, which occurred regardless, resulting in many of the HR at tSHM.

Many Edinburgh anatomists argued value of medical knowledge was greater than other ethical considerations. An article in the Edinburgh Medical and Surgical Journal from 1829 gives insight into their perspective. Firstly, it expresses shock anyone could object to the Anatomy Act given its 'great latitude' in the claimant process, and because it prevented dissection of other bodies as the (alleged) only possible avenue for obtaining 'enough' bodies.[31] The article also states, 'it is *not the dissection of one's own body, but the dissection of relatives*, which is the great object of abhorrence in the ignorant, as well as in the enlightened walks of society… which… will never in this country become different from what it is now.'[32] Of interest here is the class-coded reference of 'the ignorant' versus 'enlightened', and the assured moral emphasis upon the living, even projected into future Edinburgh. Other accounts from the period seem to reflect that 'collectors' of various kinds – from graverobbers to anatomists – were aware their actions would be morally condemned within society, given how they tried to hide them.[33] This was a complex phenomenon. While methods of obtaining HR brought disrepute upon the profession, precise pathological specimens elevated it.[34]

[28] Alberti, 2011, 74. Anatomical Bill, 1829, 212-216. Clegg, 2020, 19, 24, 61. Dittmar and Mitchell, 2016, 716. Henry, 2013, 34. Mitchell et al., 2011, 92. SHM, Edinburgh.
[29] Clegg, 2020, 20, 25. Dittmar and Mitchell, 2016, 716.
[30] Anatomical Bill, 1829, 213.
[31] Ibid., 212, 215-216.
[32] Ibid., 215-216.
[33] Clegg, 2020, 22, 24-25, 58-59, 93-94.
[34] Alberti, 2011, 71-72, 76-77. Clegg, 2020, 22, 58.

Frederick Knox, younger brother and research assistant of Edinburgh anatomist Robert Knox, wrote in the 1830s 'he who attempts to teach anatomy without a museum *bona fide* his own… strictly deserves the name of imposter… every museum takes its character… from the person who may have formed that collection'.[35] This demonstrates prestige attached to medical collections of HR, but also attitudes that the important identity was the 'collector', not the individual. It is common for HR in medical museums to be labelled with the anatomist's name, but not the individual's. This reflects how transformation of HR into medical material culture – literal objectification – began within the mind of the 'collector'.[36] Downplaying individual identity with HR also downplays potent ethical concepts attributed to personhood, such as agency and consent. The historical views of HR as property of the 'collector' contrasts with HR as property of the individual or their culture described within the previously outlined ethical discussions.

The most recent legislation applicable to tSHM is the Human Tissue (Scotland) Act of 2006, arising from the scandal surrounding collection of human tissue without consent in UK hospitals, particularly regarding organ harvesting from children.[37] This is somewhat akin to the Anatomy Act following the Burke and Hare murders. Fear of medical institutions, and imbalanced power relations, remain topical.

Case Study One: Criminal Remains

The earliest 'object' in tSHM collections is a young man's cadaver within a wooden display case, thought to be of David Myles, the first individual dissected at the Old Surgeons' Hall in 1702.[38] Aged around 19, Myles was executed for incest. The dissection – part of his punishment –was performed by the highly influential anatomist Archibald Pitcairne over a week.[39] Given the role of dissection-as-punishment, burying HR from convicted criminals could be understood as a retrojection of modern ethical norms upon Edinburgh's punishment system. A moral relativist may object to this. Alternatively, one could accept the ethical validity of using dissection as retributive or deterrent-based punishment within this society.

[35] Alberti, 2011, 65, 82-83. Alberti, 2013, 190. Dupree, 2012. Kaufman, 2001, 44. Kemp, 2009, 82-87. Knox, 1836, 5.
[36] Alberti, 2011, 5, 23, 67, 75-76, 86-87, 96-102, 212-213.
[37] Ibid., 205-206. Clegg, 2013, 55.Clegg, 2020, 20, 60, 105. Dittmar and Mitchell, 2016, 714. Henry, 2013, 40.
[38] Henry, 2013, 33-34. Kemp, 2009, 29. GC.15308, https://collections.rcsed.ac.uk/Details/collect/3702.
[39] SHM, Edinburgh.

It is worth noting incest would not be punishable by death in Edinburgh today. This crime is difficult to morally quantify. While historically, incest may have been seen as morally wrong due to Christian sensibilities, nowadays this might be codified via genetics, social contracts, or power relationships.[40] Our moral intuitions as to whether Myles 'deserved' his punishment might thus differ to other cases. For example, serial killer Burke's public dissection was ordered by a judge; tSHM has on display a pocketbook bound in Burke's skin.[41] The book reads 'BURKE'S SKIN POCKET BOOK' on the front, and 'EXECUTED 28 JAN 1829' on the back, which certainly does not invoke dignity and respect. Normally, if a museum owned a souvenir-like book bound in human skin, this would be highly controversial, let alone centrally exhibited. This example reflects both historic Edinburgh's treatment of Burke and current retributionist attitudes. Perhaps many think Burke 'deserved it': given his crimes, justice has been enacted via a fitting revenge. This highlights that historical context and attitudes of living communities affect museum practice with respect to HR.

Returning to my prior study, the identity of these HR has more recently been questioned, and the current display label does not mention Myles.[42] This case illustrates the significance of provenance in making ethical decisions for HR. If certain HR from convicted criminals should not be buried due to roles of punishment and moral judgement, then HR should be treated differently dependent on the individual's identity.

Case Study Two: Power, Class and Gender

My next case study regards two skeletons of women with osteomalacia, causing severe skeletal distortion due to vitamin D deficiency, who died in childbirth in the first half of the 19th Century.[43] Their remains were first held at the Edinburgh anatomy school of John and Charles Bell.[44] Again, identity of the anatomists is retained, not the individual. A third skeleton is from

[40] Nobel, 1978. Sebo, 2006.
[41] HC.AB.10.4, SHM, Edinburgh. The book has been confirmed to be bound in human skin (https://blogs.ucl.ac.uk/researchers-in-museums/tag/human-skin/).
[42] A portrait of Pitcairne hangs next to these HR.
[43] G.C. 13690, https://collections.rcsed.ac.uk/Details/collect/3202. G.C. 13691. https://collections.rcsed.ac.uk/Details/collect/3205.
[44] Kemp, 2009, 64.

another woman who had signs of osteomalacia in her pelvis, and died during a Caesarean section again performed by John and Charles Bell.[45]

The Anatomy Act may have accorded with justifications of Edinburgh surgeons of the value of medical knowledge versus moral goods, and the only significant harm being towards living relatives. However, given the fear the Act incited within the general population, methods of obtaining HR from the poor plausibly operated outside of moral frameworks in Edinburgh society more broadly. Museum records suggest these HR were obtained prior to the Anatomy Act, although exact dates are somewhat unclear for the first two skeletons. Details are not retained as to whether these women consented to their skeletons being displayed after death. Given their probable beliefs, it seems unlikely that they did. The dynamics of class and harmful power hierarchies between medical practitioners and the poor of Edinburgh in these cases are more striking than with HR from convicted criminals, with this method of obtaining HR less morally justifiable without recourse to punishment.[46]

These cases are more nuanced than of HR obtained in colonial contexts. Exploitation of HR from another community for profit of one's own community seems more obviously immoral than with HR obtained within the same community. For example, one could argue that research into causes of osteomalacia and its effects in childbirth could benefit future women of Edinburgh, or that their display presents opportunity for education within the local community. Also, the HR under discussion in Edinburgh do not implicate theft of cultural property as previously discussed. However, ethics of personal theft may still apply, as do MDD more generally, and the epigraph suggests medical practices in Edinburgh still entailed exploitation of one group for the benefit of another.

The component of gendered violence further complicates harmful power relations at hand. 'Specimens' of mothers and unborn children were greatly prized, and surgeons exhibited misogynistic attitudes towards women in childbirth.[47] Caesareans were frowned upon within the Christian church at the time, and often required that the surgeon choose to save the life of the mother or baby. Barilan notes this was the only procedure where dissection was performed

[45] G.C. 13689. https://collections.rcsed.ac.uk/Details/collect/3204.
[46] Alberti, 2011, 101.
[47] Ibid., 134-136. SHM, Edinburgh.

upon live patients.[48] Charles Bell's account of operation on the third individual refers to words and phrases such as 'remarkable', 'wonder', and 'a matter of admiration', showing the degree to which Edinburgh anatomists of the era objectified their patients in surgery and after death. Is it a perpetuation of harm to continue exhibiting the objectified bodies of these women, and to deny them burial? Even as their names are unrecorded, there is something deeply intimate about these remains, perhaps as the skeleton is metonymic for the individual.[49] Particularly as displays of HR for some form of disease or abnormality can resemble 'freak shows' or carnivals, there intuitively seems to be some incompatibility between displaying these types of HR and doing so with 'dignity and respect'.[50] While I cannot explore this further due to space constraints, the perhaps inevitable sensationalism and objectification within museum contexts appears to contradict the ethical valence we typically attribute to HR and the concept of the individual that persists through life and after death. How, then, can we address ethical concerns here?

Ethical Actions at tSHM

One suggestion might be burial.[51] An intriguing alternative has been explored very recently by a small number of institutions in the UK, where remains are stored in sanctified ground in relation with the Church of England. These so-called 'keeping places' mean the HR can be accessed in future if required for research purposes, therefore retaining a claim of educative value while treating the HR with an improved degree of dignity and respect to fulfil MDD.[52] This extends current DCMS guidelines, which are very general as to storage of HR.[53] Of course, this would only be an ethical solution for HR of Christian origins, leading to practical and epistemic difficulties.

In many cases of repatriation, action is dependent on the wishes of the claimant community of origin, to avoid harm to the living as previously discussed. This draws parallels with common conventions in medical ethics, akin to asking closest living relatives to give consent or make

[48] Barilan, 2006, 237.
[49] Barilan casts something akin to this as anonymisation versus depersonalisation (Ibid., 246).
[50] Alberti, 2011, 19.
[51] There are certain pragmatic considerations on volume of HR, costs of reburial, dating them, and attempting to geographically locate them in cases of poor provenance (Clegg, 2020, 141, and Cubillo, 2010, 24-25).
[52] Clegg, 2020, 5, 141-143.
[53] Ibid., 72, 75-76, 82.

medical decisions when the individual cannot.[54] Accepting this suggests the relevant community (Edinburgh) should be surveyed as to what should be done with these HR. This could follow Historic England (2009) and Cambridge County Council (2009) surveys, which generally found people in the UK expect historic HR to be held in museums for display and research.[55] Responses in the visitor book at SHM are largely of two strands: expressions of educational value ('fascinating', 'interesting'), or of disgust ('yuck', 'gross', 'how am I going to have lunch now?'). However, I found no responses expressing sadness or ethical concern, suggesting display of these HR does not constitute ethical harm to living observers. This may be because the visitor book does not survey the appropriate community. The Christian community in Edinburgh could arguably help curators clarify ethical norms regarding Christian HR. Clegg, former head of the Human Remains Unit at the Natural History Museum, stresses the need to only consult people who are 'genuinely connected' with HR in cases of repatriation.[56]

One highly intuitive ethical judgement is that HR should only be used if there is good justification for doing so.[57] HR provide opportunities for medical students to view parts of the body that would otherwise be near impossible, in a more permanent form. It appears difficult to deny educational value of HR at tSHM. They present clear, visual opportunities to witness variations within human bodies without formal medical education, and are accompanied by long, detailed texts. Many medical collections became less available to the public by the mid-century.[58] It seems undesirable, and perhaps an extenuation of historic power relations, that only medical practitioners could access these collections, which I believe extends to the present day.[59] Museum practitioners today often retreat to this moral 'high ground' of public education, perhaps in contrast to historical emphasis on medical research.[60] This suggests tension between moral duties towards the dead and the living.

[54] Alberti, 2011, 58.
[55] Clegg, 2020, 88, 137.
[56] Ibid., 151.
[57] Barilan, 2006, 245.
[58] Alberti, 2011, 173, 194, 202.
[59] Henry, 2013, 42.
[60] Further work is required to more fully address questions of museums for education versus for entertainment and spectacle.

Clegg states that HR should only be displayed when necessary in order to 'educate and inform', and 'never… to shock, titillate or make a display 'sexy' or scary''.[61] To what degree is this possible? Visitor responses are variable, and strongly dependent on historical and social contexts. For example, text accompaniments provide additional information on HR, demonstrating willingness on the part of the museum to act as imparters of knowledge. However, it is up to the visitor to read them, and Samuel Alberti – Director of Collections at National Museums Scotland - suggests they do little to mitigate emotive reactions of 'wonder, horror and disgust'.[62] Modern visitors might generally be expected to experience more discomfort and squeamishness around HR.[63] One could argue this modern disconnection with dead bodies is precisely why it is important for museums to provide spaces where visitors can confront the realities of death and the human body. Many visitors are particularly drawn to HR within exhibits.[64] One might also assert these emotional responses are desirable, in that they encourage visitors to attend exhibitions and gain knowledge.

A related argument in favour of continuing to display all cases of HR at tSHM is that removing them might appear somewhat a rewriting of history and denial of former harm. Alberti notes changing emphasis at tSHM towards 'heritage', and curators at tSHM purposefully retain the museum's historical interior.[65] Material from tSHM website similarly emphasises historical education.[66] The tSHM Anthology reads that HR provide '…a physical representation of the development of ideas, a window to imagining the past and how people who have gone before have shaped our lives today… [t]hey teach us compassion and gratitude, and help us know what it is to be human, and to be alive'.[67] This implies some sense of *moral* education, whereupon displaying these HR is a moral good. For example, the Burke pocketbook is demonstrative of retributionist and deterrent modes of punishment, whereas the female skeletons illustrate harmful medical and societal hierarchies. Also, as previously alluded, HR could help visitors develop more ethical attitudes towards death and the human body via exposure to its realities. Museums thus operate as institutions with moral authority. For this argument to hold, the museum would need to draw sufficient attention to former harms, power

[61] Clegg, 2020, 77, 85, 88.
[62] Alberti, 2011, 4, 24, 154-156, 163-164, 176-177, 185-191, 193-194, 209.
[63] Clegg, 2020, 8, 88.
[64] Ibid., 151.
[65] Alberti, 2013, 195. Henry, 2013, 32.
[66] https://museum.rcsed.ac.uk/wohl-pathology-museum [Accessed: 12/02/2023].
[67] Kemp, 2011, 7, 11.

relationships, and ethical considerations. Doing so could help fulfil MDD and their own duties as moral authorities and disseminators of knowledge.[68]

This leads to the main conclusion of this essay. In my opinion, tSHM does not currently fulfil this requirement. One reason is the use of literally objectifying language of HR as 'objects', which occurs in text accompaniments throughout tSHM. I argue this should be changed, to humanise these individuals. It was a plausibly immoral practice in the past for collectors to impose their identities upon HR and demand an ontological switch for HR into mere objects, especially given relatively close temporal links between these HR and living communities, so museum practitioners should avoid perpetuating this objectification even if harm to living communities is less pronounced in the present. Similarly, many HR are accompanied by passive descriptions that they 'come from a female' or 'became part of' the collections, without information about how they were obtained. This deemphasises the role historical agents played in causing harm to these individuals. Similar changes in museum language practice are being made in other spheres, most prominently in decolonisation efforts (e.g. Prianti and Suyadnya, 2022. Giblin, Grout and Ramos, 2019). Other than some small text accompaniments within the History of Anatomy exhibition space, there is relatively little about how these specimens were obtained (particularly in the Pathology Museum), and about historic, harmful power relations in Edinburgh. Similarly, there is no content on ethical decision making performed by museum practitioners. Each case must be precisely historicised and contextualised to clarify differences between HR from a poorhouse and a serial killer. In this way, SHM fails to utilise the argument that to remove HR from display is to rewrite history, and to display them is a moral good, as they are not sufficiently explaining and representing the relevant history.

Conclusion

Within this essay, I have identified a gap in the ethical and museological literature for HR that are kept in museums within the community in which the individual lived, particularly for medical museums. While there is still need for developing the best ethical framework regarding HR, I have preliminarily analysed relevant ethical considerations of property, consent and means-to-ends, clarifying some ethical issues currently underdiscussed yet implicit in existing

[68] This illustrates important differences in displaying HR versus facsimiles of HR, which may otherwise defeat medical education arguments.

museological practices and literature. I have also justified the importance of history for ethics relating to HR, explored an original set of case studies via tSHM in Edinburgh, and provided pragmatic suggestions for ethical action. At the crux of this essay, I have proposed that regardless of decisions on whether to retain or display HR, museum practitioners at tSHM should revise their use of passive, objectifying language, and provide more information on the historical power relationships and methods of acquisitions that led to these HR being held at the museum. Doing so is in accordance with MDD, to treat HR with dignity and respect, and to maximise the goods of displaying HR for medical, historical and moral education.

I have mainly addressed cases of skeletons, but most HR at tSHM are pathology specimens. These are more ethically complex (for reasons such as: reburial would be insufficient for resurrection, historicization is more difficult, and they are less metonymic for the human), so further research is required, but my arguments on contextualising and deobjectifying HR at tSHM plausibly still hold. While I have looked at tSHM, recent changes in museum practice regarding HR bear mentioning: the body of Charles Byrne has been taken off display at the Hunterian Museum in London, the Medicine Man permanent exhibition at the Wellcome Collection has closed, and the Pitt Rivers museum has removed its collection of shrunken heads from display.[69] Future work could involve comprehensively examining and comparing duties towards HR at different institutions. Historical specificities of each case are important for ethics across the spectrum of HR.

[69] Board of Trustees of the Hunterian Collection, 2023. Wellcome Collection, 2022. Pitt Rivers Museum.

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Appendix

1: Museum Text Accompaniment, GC.153

Cadaver
Scottish, 1702

This body of a dissected child was presented to the College in 1702 by Archibald Pitcairne. This is the earliest remaining object in the collection. It was donated shortly after the Incorporation advertised for items for is museum in 1699. Pitcairne presided over both of the public dissections at Old Surgeons' Hall in 1702 and 1704.

2: Museum Text Accompaniment, GC.13690 and GC. 13691

Skeletons with severe osteomalacia
1825

Osteomalacia is the adult form of rickets, a condition that causes abnormal bone development resulting in weakened, soft bones. It is most commonly caused by a lack of calcium and Vitamin D. Today, it would be routinely treated by making changes to the person's diet and possibly taking vitamin and mineral supplements.

These two very severe cases are both women who died during childbirth, having had rickets since childhood . In the early 19th century the link between diet and the condition was unknown and the condition would progressively worsen over time, as can be dramatically seen here.

The skeleton on the left is from a woman who was well known around the city. In the contemporary notes she is described as 'the little woman, with rather a good face and a great deal of impudence, who generally stood at the Infirmary Gate to talk to the students. She went with great velocity on her crutches; she had no motion in her legs separately, they became twisted together curiously.' She died during the second day of labour due (sic) a ruptured pelvis. Charles Bell, who attended her, notes that this was brought about by complications

caused by the distortion of her spine. The woman had lost a child late on in a previous pregnancy, again thought to be down to a complication of her rickets.

3: Museum Text Accompaniment, GC.13689

Skeleton
1800

This skeleton belonged to a woman from Penicuik, Scotland. She had one of the earliest known recorded Caesarean sections in Britain. She suffered from osteomalacia, a softening of the bones due to lack of vitamin D. This affected her pelvis and made it difficult for her to have children. She became pregnant six times and lost all of the babies. During her seventh pregnancy, doctors confined her to her bed. In 1800, she arrived in Edinburgh and consulted her previous surgeon, Doctor James Renton. He contacted John Bell - a well-known surgeon. Bell, his brother Charles, and Renton made an incision into the mother 's abdomen and into the uterus. Her child survived but unfortunately she died twenty minutes later from blood loss. The boy is thought to have been named Caesar. This skeleton became part of the Extra Moral Anatomy School at Great Windmill Street in London. It was sold to the College in 1825.